NORTHSTAR HOME

Customer Care Enablement

WN7 | Needs Assessment & Planning

Performance Analysis, Design Rationale & Planning Organizer

> A structured record of the needs assessment and planning work informing the proposed WN7 performance-support strategy.

01 Lead alignment

What leads know

- (Experienced leads-- not all leads) Awareness of the problem
- Existence of available resources/tools (not necessarily the "when" and "how" behind them, along with smaller distinctions, and reliably knowing their locations)

What leads need

- "When I coach, I need a before and after. What did the customer say, what did the Guide do, and what should they try next time? I can do that in five minutes if I have a clip and a cue."
- "But I could not tell them which Beacon article to use. I wrote the temporary specialist page name on a whiteboard, and it is not visible to everyone."
- Cues-- "Leads need a tiny cue for what to listen for."
- Common requests were: a one-page coaching cue, one strong and one weak call excerpt, a clear answer for pending Plus status, and a way to tell whether the Guide had actually used the serial screen.
- "Seven respondents said they had shared the Beacon article. Four said they had run a huddle or planned one. Three said they had used a call example. Several said this was harder to track than an assigned Sana module because shifts and local coverage differ."

Team leads working meeting

Meeting agenda

- Situation brief and WN7 refresh (3 mins)
- Case Review & Coaching Alignment (19 mins)
  - Discuss the cases (9 mins) — What happened and why?
    - Case A: Strong recovery with a Plus complication
    - Case B: Serial checked, but generic troubleshooting continued
  - Align on expectations (10 mins) — What should Guides do, and how should Leads coach or escalate?
- Takeaway resource, expectations, next steps (3 mins)

Meeting Objectives

- **Establish understanding**
  - Refresher on the basics
    - What's the situation? What has been noted?
    - Current state of things/needs
    - Biggest hangups/hiccups observed so far
      - Hiccups with Plus get a mention here
    - Mention the beacon article, search troubles
- **Establish alignment**
  - **Coaching outcome:** Leads can recognize where Guide execution broke down, investigate whether the cause is a knowledge gap, Orbit interpretation issue, or access/process blocker, and determine the appropriate coaching or escalation response.
  - Specialist-supported small-group case discussion, followed by shared alignment on Guide expectations and coaching responses.*

    **Part of the continuous 19-minute case discussion and alignment, not an extra activity. Specialist availability is a staffing dependency (needs Specialist Care sign-off), not a confirmed commitment.*
  - Agree on the Guide recovery sequence, what Leads should reinforce in huddles, and how to handle or escalate unresolved Plus questions.

- Confirm where to find BH-4419 and the Orbit serial tool; clarify when Guides should use each.
- **Agree on coaching cues**
  For each cue, discuss what Leads should listen for, what they should check, and whether the appropriate response is coaching or escalation.

Coaching cue	Example customer language / call evidence*	What Leads should investigate
WN7 recognition	• *"Everything dropped"* • *"My whole house went offline"*	Did the Guide recognize the complaint pattern and move toward eligibility verification?
Serial verification	• Orbit serial check recorded	What result did the Guide see and understand?
Recovery decision	• Eligible result recorded • Generic troubleshooting continued	Why didn't the next action match eligibility?
Complete recovery	• Order completed • Plus unresolved	Was follow-up owned and explained?
Customer explanation and documentation	• Prior notes didn't clarify whether an order or credit had started	Could the customer and next Guide understand what happened?

**Examples are drawn from documented Northstar language or call patterns, not necessarily verbatim customer recordings. System records alone don't establish Guide understanding. These are proposed coaching aids, not new mandatory QA criteria.*

Lead response: Identify the breakdown, ask the Guide what happened, coach the behavior when appropriate, or escalate a tool, access, or process blocker.

Takeaway resources for Leads

- **Lead one-pager** — Operational reference
  - Brief WN7 situation summary and key reminders
  - Expected Guide recovery sequence
  - Where to find BH-4419 and the Orbit serial tool, and when to use each
  - Lead responsibilities, follow-up expectations, and escalation channels
- **Lead Coaching Cues** — Coaching reference
  - Strong and weak call examples*
  - Observable cues: what to listen for and what to check
  - Questions to help diagnose execution breakdowns
  - Suggested coaching responses versus issues requiring escalation

**Use approved, de-identified call excerpts where available. Otherwise, use clearly labeled illustrative scenarios grounded in documented patterns and validated by Quality/Specialist Care.*

Moving Forward

- Shared Lead message thread — a participating Lead designated by Care leadership serves as temporary thread Admin during rollout.
  - Purpose: Keep Leads, Specialist Care, and Quality aligned as WN7 situations unfold. Use the thread to ask questions, flag recurring Guide difficulties or tool blockers, share coaching observations, and communicate verified updates.
- Actions for Leads afterwards
  - **Introduce and reinforce Guide practice:** In the team huddle, use the sequence slide to introduce the WN7 workflow, open dialogue with Guides, and launch the individual scenario practice. Show Guides where to access it through Sana, where to find the Call Path, and the video resource if approved. Reinforce the same decision sequence through later coaching and call reviews.
  - Use the agreed coaching cues during call reviews and Guide escalations. Surface recurring problems or blockers through the shared lead thread
  - **Monitor and coach Guide execution:** During existing call reviews and coaching opportunities, Leads look for observable evidence that Guides recognize the WN7 pattern, verify eligibility, choose the appropriate path, complete or arrange follow-up, and explain/document the outcome. Diagnose breakdowns, provide targeted coaching, and escalate tool or process blockers.*

    **Leads verify observed execution in the calls they review, not proficiency across every Guide.*

02 Equipping Guides

What Guides need

- Guides need to locate the correct guidance quickly without interrupting the customer interaction to search through Beacon.
- Understanding of Plus-- what it does & doesn't do, transaction status, steps
- Guidance for unavailable Orbit access, missing serial information, or results the Guide cannot interpret, including when to involve a Lead.
- **Customer Communication** — Know what to say and how to move the call forward
  - Guides need concise, actionable language for transitioning from the customer's complaint into verification and the appropriate next step.
  - Customer-facing language for acknowledging disruption, explaining the next step, and setting accurate expectations.
  - Guide feedback: “a few words that get me to the next action” and “I need to know what I click and what I say...”
  - Possible customer-facing language: “We found an issue with some units, and I can check whether yours is included.” Requires approval.
  - **Practical verification prompt:** Sofia suggested asking customers to check the label on the underside of the main unit. One reported example helped identify a WN7. Requires procedural verification before use.
- **Cues** — Recognize when to act
  - *"If a customer says the whole house went down and then it came back, I do not know if that is their modem, the weather, or the WN7. I ask them to reboot because I need somewhere to start. If I skip it and it is a normal connection issue, I get asked why I did not use the basic steps."* (Guide M., Sep 05)
    - Guides may default to generic troubleshooting when they're unsure whether WN7 eligibility verification applies. Standard troubleshooting isn't wrong in itself, and it remains the right path when the pattern or serial doesn't fit.
  - Ben Cruz's suggested recognition cue: *“Whole home, brief drop, returns on its own, then model and serial.”*
  - Imani's refinement: Listen for and acknowledge the customer's impact before moving into verification.
  - Important distinction: Recognizing the complaint pattern should trigger eligibility verification, not an assumption that the customer automatically qualifies for replacement.
- **Proper Sequence** — Know what to do next
  - Recognize → Verify → Decide → Complete → Explain & Document
  - **Verification support:** Help Guides locate the device model and serial, use Orbit to check eligibility, and follow the appropriate recovery or standard-support path.
  - The proposed Guide Call Path will support this sequence with concise action prompts, decision points, and direct links to Orbit and the authoritative Beacon guidance.
  - The sequence must account for eligible, ineligible, and unclear cases, including unresolved Plus transactions and appropriate follow-up or escalation.
- **Knowing when recovery is complete**
  - Understand that placing a replacement order doesn't necessarily close the recovery if an applicable Plus action remains unresolved.
  - Know how to confirm completed actions, identify unfinished work, arrange or escalate follow-up, and document the status for the next contact.
  - Explain clearly to the customer what's been completed, what's still pending, and what happens next, using approved guidance.

Working design notes, not learner-facing instructions. Ben's cue (Sep 10) and Imani's refinement were suggestions from a Team Lead preparation call; no wording was approved for callers, and neither is an approved Northstar procedure. Cues help Guides recognize when to act; the sequence helps them know what to do next.

Guide actions

1. Attend the team huddle— Review the WN7 update, discuss the decision sequence with the Lead, and see the supporting slide and available resources.

2. Locate and review the Guide Call Path — Understand the five-step sequence and where to access it during live calls.

3. Complete the WN7 practice scenario* *(proposed)* — Work through a short, individually assigned digital scenario during scheduled work time. Write what you would do and say, then compare your responses with approved model answers. The scenario still needs development and content validation before launch.

4. Confirm live support route — Before live WN7 calls, identify how to reach your assigned Lead during a call and which established escalation path to use if they're unavailable.

5. Escalate unresolved issues — When a problem comes up on a call, bring unclear eligibility, missing information, tool access problems, or unresolved Plus actions to the appropriate Lead or support team.

6. Apply the Call Path during live WN7 contacts — Verify eligibility, select the appropriate action, complete or arrange follow-up, and explain/document the outcome.

7. Participate in follow-up coaching — Bring one decision, question, or uncertainty from the practice scenario to your next coaching conversation with your Lead. Address additional gaps identified through call reviews or observation.

Pending Approval / Readiness

8. Watch the short Orbit serial-tool video *(optional)* — Supporting resource, conditional on approved screen access, validated procedures, and production. Available through the same launch communication if approved. Rollout doesn't depend on it.

9. Locate the Plus FAQ in Beacon *(pending approval)* — Conditional on content approval and publication. The FAQ itself is pending, but an authoritative Plus support route is required either way (see Rollout Dependencies).

Rollout Dependencies

**Items marked with an asterisk are related to or mentioned in the dependencies below.*

- Sana is the proposed assignment/access point for the scenario, pending Learning Operations confirmation of delivery capabilities.
- Workforce must approve scheduled practice time. Care leadership and Workforce must also confirm Lead availability for live support and an established fallback route.
- **Required regardless of FAQ status:** an authoritative Plus guidance or escalation route Guides can use before launch.

Guide resources

The **Guide Call Path** is the central resource. The others introduce it, practice it, or provide deeper authoritative guidance.

Resource	Primary job
Guide Call Path	In-call decision and execution support
Team huddle slide	Introduce and visualize the sequence
Digital practice scenario	Rehearse decisions and customer language
Beacon Plus FAQ *(pending approval)*	Authoritative Plus answers
Orbit video *(optional)*	Demonstrate serial-tool navigation

The live support route (#4) is an operational arrangement, not a resource.

03 Rollout & Reinforcement

Rollout Success

Defining success across Guide execution, customer outcomes, and operational capacity

Reviewed WN7 calls show more consistent recognition, eligibility verification, appropriate recovery decisions, and clearly documented follow-up than the pre-rollout baseline. Customer outcomes show promising improvement, while service capacity remains manageable.

Stakeholder Evidence

> "Once the path is live, the important questions will be: did they recognize the pattern, check eligibility, complete both transactions, and explain what happens next?"

Malik, Quality reviewer, Sep 15 calibration comment

Observable Indicators of Successful Execution

Defining the behaviors Leads and Quality look for when reviewing WN7 calls

Dimension	What reviewers look for
Recognition	Recognizes the potential WN7 pattern and moves toward verification
Verification	Checks eligibility and correctly interprets the result
Decision	Selects the appropriate path based on verified eligibility
Recovery	Completes both applicable transactions—replacement and Plus—or establishes documented follow-up for unresolved work
Communication	Accurately explains what was completed, what's pending, and what happens next
Documentation	Records completed actions, outstanding work, and follow-up ownership

These indicators guide review of relevant behaviors in baseline and pilot calls. Apply them according to the circumstances of each call, distinguish observed behavior from system records, and investigate gaps before drawing conclusions.

Rollout Framework

Designed backward from the success criteria above

Each phase should answer: *Does this phase make the desired Guide behavior possible, support its execution, or provide evidence that it's happening?*

Phases advance on evidence, not on a schedule. Each one ends at a decision point: proceed, adjust, or pause. Care leadership makes each call, informed by Quality, Workforce, Learning, and Specialist Care (see 04).

*Working framework, not the finished rollout plan. No dates, numeric targets, staffing commitments or sample sizes. Unresolved items are labeled **Dependency** or **Open decision**.*

1. Prepare

Establish operational readiness

Purpose	Ensure approved guidance, tools, ownership, Lead alignment, Guide support arrangements, and Workforce capacity are ready before introducing the intervention.

Key activities	• Confirm the Guide Call Path content against BH-4419 and approved guidance. • Put approved Plus guidance or an authoritative escalation route in place (**prerequisite**). • Confirm where the eligibility result appears in Orbit and that General Care has access (**Dependency:** Care Systems). • Develop and validate the digital scenario; confirm Sana can assign it natively or by link (**Dependency:** Learning Operations). • Run the Lead working meeting (01) for the pilot teams' Leads. • Confirm Guide → Lead live support and an established fallback route; Specialist participation only as capacity allows (**Dependency:** Care leadership, Workforce, Specialist Care). • Workforce selects the pilot teams and Guides and schedules practice time, accounting for queue and Specialist strain (see Pilot). • Care leadership designates a participating Lead as the temporary thread Admin; the specific person is confirmed here (see 04). • **Baseline comparison approach:** before launch, Quality sets how baseline and pilot calls will be compared (see the comparability note under Observe & Adjust). The established pre-rollout findings are the 204 eligible General Care calls reviewed Sep 21–30: 76 complete recoveries (37%), 13 incomplete (6%), 115 generic troubleshooting (56%). These aren't scores against the six indicators. Revisiting the underlying calls against the six indicators, where possible, isn't a launch prerequisite; it must be complete before pilot results are interpreted. • Orbit video: optional, not a prerequisite.
Evidence / outputs	• Each readiness item confirmed by its owner, or flagged as missing. • Comparison approach set by Quality; established baseline findings on hand. • Pilot teams' Leads aligned on the recovery sequence and coaching cues.
Decision point	• **Proceed:** every required item is confirmed: ◦ Approved Call Path and authoritative Plus guidance or escalation route ◦ Validated Orbit eligibility workflow and General Care access ◦ Confirmed scenario delivery/access through Sana or an approved link ◦ Lead alignment and a functioning Guide → Lead support/fallback route ◦ Workforce-approved pilot group and scheduled practice time • **Adjust:** only optional items are outstanding (e.g., the Orbit video). Launch without them and label them pending. • **Pause:** any required item is unconfirmed. Required items aren't bypassed or worked around.

2. Pilot

Introduce and practice the intervention

Purpose	Roll out the complete intervention to a limited, Workforce-approved group: Lead alignment, Guide team huddle and slide, Call Path, individual digital scenario practice, and live support.
Key activities	• **Proposed pilot design:** 2–3 Lead teams with a representative mix of Guides, not volunteers or only high performers. Workforce determines the final Guide count, team selection, and scheduling based on operational coverage. Not an approved staffing commitment. • Leads run the team huddle with the sequence slide, introduce the Call Path, and launch the scenario. • Guides complete the individual scenario during scheduled work time, through Sana (pending confirmation). It replaces only the previously proposed micro-huddle. • Guides apply the Call Path on live WN7 contacts right away, with Guide → Lead as the primary support route. • **Observation window:** Quality can begin observing and reviewing live WN7 calls as soon as they occur. Before drawing performance conclusions or recommending expansion, Guides need sufficient live-call exposure and Quality needs sufficient comparable evidence. The length isn't specified here. • Leads bring questions and blockers to the shared Lead thread, where the Admin routes them. The thread coordinates Leads; it doesn't replace live Guide → Lead support or established escalation.

Evidence / outputs	• Huddle attendance and scenario access/completion recorded. These are delivery measures, not evidence of proficiency. • Questions, blockers, and live-support requests logged. • Early observations of live WN7 calls, held for interpretation in Observe & Adjust.
Decision point	This gate checks whether the intervention is reaching Guides and its components are usable. Whether Guide execution is improving is evaluated in Observe & Adjust. Delivery measures such as scenario completion never stand in for demonstrated execution. • **Continue:** each component is reaching Guides and is usable as designed. Observation continues; performance conclusions wait for sufficient live-call exposure and comparable evidence. • **Adjust:** a component isn't landing (e.g., scenario access, Lead availability); fix it before drawing conclusions about execution. • **Pause:** queue or Specialist capacity becomes unmanageable, or Guides lack a working support route.

3. Observe & Adjust

Evaluate execution and identify breakdowns

Purpose	Use the six observable indicators to examine real WN7 calls. Compare against baseline evidence, investigate gaps, gather Guide/Lead feedback, and refine guidance or support as needed.
Key activities	• Quality completes the needed baseline review before interpreting pilot results. • Quality leads the evaluation of pilot WN7 calls against the six indicators, using the same criteria as the baseline. Leads contribute coaching observations and help diagnose gaps; Workforce provides capacity data. • Diagnose breakdowns before coaching: next step unknown, Orbit result misread, access or process blocker, perceived pressure to keep troubleshooting. • Coach, or escalate tool and access problems to their owners. • Gather Guide and Lead feedback; review recurring issues in the Lead thread. • Refine the Call Path, scenario, or coaching cues where evidence shows a gap. • Read the evidence in three distinct layers: ◦ **Guide execution:** the six observable indicators. ◦ **Customer recovery outcomes:** complete and incomplete recovery; repeat contacts, where measurable. These show whether individual customers received the right resolution. ◦ **Operational signals:** average handle time; wait time and abandonment; Lead and Specialist support load. Useful for capacity, but they don't show whether a customer received the right resolution. • Recovery and operational measures are proposed (**Dependency:** Quality and Workforce confirmation).
Evidence / outputs	• Indicator-based observations from reviewed pilot calls, compared with the baseline. • Documented breakdown causes and the response to each. • A log of refinements made. • A readout of customer recovery outcomes and operational signals, read alongside the call reviews. Changes observed during the rollout don't by themselves show the intervention caused them. • Partner recommendations to Care leadership (see 04).
Decision point	• **Proceed to Expand:** based on sufficient live-call exposure and comparable evidence, there is meaningful improvement in observable Guide execution, critical workflow blockers are resolved, and operational capacity is manageable. Customer recovery outcomes show promising directional evidence; not every operational signal needs to improve right away. • **Adjust:** improvement is limited or mixed, or gaps trace to fixable causes. Refine and observe again. This is the usual response. • **Pause:** reserved for serious operational strain, guidance that proves inaccurate or unusable, unresolved critical access or support problems, or other conditions that make continued rollout inappropriate.

Comparability: *baseline and pilot reviews use comparable call-selection approaches, the eligibility branches that apply to each call, the same six indicators, and an explicit, consistent treatment of missing evidence. Review sampling and cadence aren't specified in this proposal.*

4. Expand

Scale based on evidence and capacity

Purpose	Extend the intervention beyond the pilot only when observed execution, resource readiness, and operational conditions support expansion.
Key activities	• Expand in successive waves of Lead teams, not one organization-wide release. • Care leadership decides whether evidence supports expansion; Workforce approves each wave's composition, timing, scheduled practice coverage, and operational feasibility. Both are required before a wave proceeds. • Each Lead is aligned before their Guides enter the rollout. This doesn't require all 18 Leads to take part before the initial pilot. • Repeat the Prepare checks for each new wave (Lead alignment, support route, access). • Carry pilot refinements into the Call Path, scenario, and coaching cues. • Keep observing with the same six indicators.
Evidence / outputs	• Readiness confirmed for each new wave. • Execution observed holding as waves are added. • Capacity readout for each wave.
Decision point	Each wave goes through the same evidence gate as the pilot. • **Proceed:** Care leadership finds the evidence supports expansion and Workforce approves the wave. • **Adjust:** new waves show different breakdowns; slow down and address them. • **Pause:** queue or Specialist capacity is strained.

5. Sustain

Maintain execution and reinforcement

Purpose	Continue targeted Lead coaching, monitor recurring problems, maintain authoritative guidance, and revisit the intervention when procedures or operational conditions change.
Key activities	• Leads reinforce execution through existing call reviews and coaching, using the agreed cues. • Quality monitors recurring execution gaps through existing reviews and shares findings. • Learning updates the Call Path, scenario, slide, and Lead resources when BH-4419, Orbit, or Plus handling change, once the relevant owner has validated the change (see 04). • Wind down temporary rollout support, such as the thread Admin role and Specialist participation in the thread, when normal coaching and escalation are enough.
Evidence / outputs	• Continued indicator-based observation in routine call reviews. • Guidance and Call Path updates logged with an owner and review date.
Decision point	• **Maintain:** execution holds through routine coaching. • **Revisit:** procedures change, or recurring breakdowns reappear. • **Retire:** temporary supports, once routine coaching and escalation are enough.

04 Ownership

Ownership & Decision Authority

Who owns the work, who provides evidence, who validates procedures, and who authorizes progression

Proposed responsibilities requiring organizational confirmation, not existing commitments.

Rollout Decision Authority

Care leadership owns the final proceed / adjust / pause decision at each gate, informed by:

Partner	Contribution
Quality	Evaluates Guide execution against the six indicators, compares baseline and pilot findings, identifies recurring gaps, and recommends whether performance supports expansion.
Workforce	Evaluates staffing coverage, queues, wait times, abandonment, and scheduled practice capacity; approves each wave's composition, timing, practice coverage, and operational feasibility.
Learning	Confirms instructional readiness, examines practice and coaching feedback, identifies design changes, and recommends refinements.
Specialist Care	Validates recovery procedures, identifies complex recovery issues, and evaluates whether Specialist support remains sustainable.
Care leadership	Weighs the evidence, resolves competing priorities, and authorizes proceeding, adjusting, or pausing.

Expansion requires both: Care leadership decides that evidence supports expansion, and Workforce approves the wave's composition, timing, scheduled practice coverage, and operational feasibility.

Accountability by Area

Area	Owns	Provides evidence / input	Validates or authorizes
Rollout progression	Care leadership	Quality, Workforce, Learning, Specialist Care	Care leadership authorizes; each wave also needs Workforce approval
Pilot and wave composition	Workforce (composition, timing, scheduled practice coverage, feasibility)	Coverage and capacity data	Workforce approves each wave
Evaluation	Quality	Leads (coaching observations, gap diagnosis); Workforce (capacity data)	Recommendation to Care leadership
Guide learning materials (Call Path, scenario, slide)	Learning	Quality findings; practice and coaching feedback	Specialist Care / policy owners (procedures, Plus); Care Systems (Orbit)
Lead resources (one-pager, Coaching Cues)	Learning (production and maintenance)	Quality findings; Lead feedback	Quality and Specialist Care (relevant operational content)
Scenario delivery (Sana)	Learning Operations (assignment, access, delivery, native or via approved link)	Learning (scenario content)	Learning Operations confirms

Area	Owns	Provides evidence / input	Validates or authorizes
Lead working meeting	Learning (facilitates)	Care leadership ensures participating Leads can attend	Specialist Care supports only with approval (see below)
Live Guide support	Care leadership (support arrangement)	Leads provide first-line support; established fallback route stays in place	Workforce validates coverage; Specialist Care confirms escalation availability, subject to capacity
Recovery procedures and Plus handling	Specialist Care / appropriate policy owners	Complex recovery issues from the field	Validated within their authority; published in Beacon
Orbit access and tool behavior	Care Systems	Access and tool blockers raised by Leads	Care Systems validates
Shared Lead thread	Temporary Admin (a participating Lead, confirmed during Prepare)	Lead questions and blockers	Care leadership designates the Admin
Reinforcement	Care leadership and Leads	Call reviews and coaching	—

Specialist Capacity Approvals

Mateo (Specialist Care) approves Specialist involvement separately for:

- (a) the Lead working meeting, and
- (b) temporary shared-thread support.

Neither approval implies the other.

Shared Lead Thread Administration

Care leadership designates a participating Lead as the temporary thread Admin; the specific person is confirmed during Prepare. The Admin:

- Coordinates questions and routes blockers to the appropriate owners.
- Tracks unresolved issues and recurring patterns.
- Circulates verified answers to participating Leads.
- Doesn't personally own every procedural resolution.

The thread is for Lead coordination. It doesn't replace live Guide → Lead support or established escalation routes, and it isn't a permanent new support channel. Specialist participation stays conditional on capacity, with no guaranteed real-time response.

Sustainment Ownership

Instructional maintenance is separate from operational accuracy

- **Learning:** maintains the Guide Call Path, digital scenario, huddle slide, Lead one-pager, Coaching Cues, and associated learning materials.
- **Quality:** monitors recurring execution gaps through existing reviews and shares findings for coaching or resource updates; validates relevant operational content in the Lead resources, with Specialist Care.
- **Care Systems:** validates Orbit access, tool behavior, and relevant system instructions.
- **Specialist Care / appropriate policy owners:** validate recovery procedures and Plus handling within their authority.
- **Care leadership and Leads:** reinforce approved guidance through team communication and existing coaching.
- **Beacon:** remains the authoritative location for approved procedures.

Learning doesn't independently approve operational policy.